Royal Devon University Healthcare NHS Foundation Trust

Standard Operating Procedure (SOP) for Major Haemorrhage in Community Hospitals

Summary

Major haemorrhage in community hospitals is a rare event.

This SOP will assist with obtaining blood components peri-operatively in an emergency at community hospitals and with the communication and coordination of patient care.

Publication Details

Authors of Clinical Guideline	Dr Nim Pathmanathan (Consultant Anaesthetist) Dr Daryl Johnston (Consultant Anaesthetist) Dr Jennifer Davies (Transfusion Laboratory Manager)
In consultation with:	
Division responsible for Pathway	Surgery
Contact details	Nim Pathmanathan Department of Anaesthesia
Version number	1.0
Replaces version number	New
Date written	5th February 2025
Approving body and date approved	Anaesthetic Governance East 11th November 2025 Pathology Governance 18th November 2025
Review date (3-6 months prior to expiry date)	5th September 2027
Expiry date	4th February 2028
Date document becomes live	12th February 2026

Locations covered: Heavitree, Tiverton and Exmouth hospitals

In the event of a major haemorrhage peri-operatively at one of these locations:

1. **Activate major haemorrhage protocol and state location:**

   - Call 01392 402222 'I want to activate the major haemorrhage protocol at ….'

   Bleep will be disseminated to

   - Transfusion lab
   - Duty Floor Anaesthetist (DFA) # 489 (Consultant on call after 17:30)
   - Operating department practitioner (ODP) bleep holder # 415

   Telephone transfusion laboratory staff using mobile 07920 744041,state that this is a major haemorrhage in **xxx community site (state the name of the site)**

   Transfusion will prepare pack 1 (4 x RCC (Red Cell Concentrate) and 2 x FFP (Fresh Frozen Plasma) – see flowsheet below).

   4 RCC O D-negative (O neg) will be available immediately and 2 FFP (group A or AB) will be available within 20 minutes.

2. **In addition, contact the DFA directly on:**

   - Duty anaesthetist phone in anaesthetic office **01392 402296**
     or duty mobile **07423555113**
     or consultant mobile via CLW
     or **bleep 489**

   - Or (if DFA unavailable or after 17:30) Theatre 9 consultant **01392 402409**
     or mobile via CLW

   Explain nature of critical incident – confirm transport of blood, equipment and assistance to community hospital is the preferred plan rather than transport of patient to RD&E.

   Consider what other equipment/ assistance may be required, e.g.:
   - Ranger (plus insert) and blood giving set
   - Transport monitor + leads and transport ventilator (from ICU)
   - Arterial line, transducer and pressure bag
   - Any emergency drugs not available at community e.g. tranexamic acid (TXA), calcium
   - Surgical assistance

3. **DFA to liaise with ODP bleep holder**

   - DFA identifies anaesthetic/ critical care consultant to attend community hospital – **a consultant with a car on site and business insurance to be covered for the trip to the community.**

- The DFA may be most suitable person, or require an elective list to be paused or swapping with an anaesthetist with an ongoing list.

- Handover DFA bleep to Theatre 9 consultant if no other consultant is free.

- If surgical assistance is required, liaise with on call surgical specialty and consider transport together to community hospital (if appropriate) and if any extra surgical equipment is required.

- Consultant anaesthetist to pick up car and park at oncology entrance and attend transfusion lab to collect blood components, which will be in 2 sealed transport boxes (A2 template 1st floor above oncology entrance). Bleep 227 if out of hours and no laboratory staff in transfusion (telephone in the blood fridge room next to the laboratory)

- The blood component transport boxes can store components for up to 4 hours from the packing time stated on the tag attached to the box. Do not transfuse components from boxes over the 4-hour period.

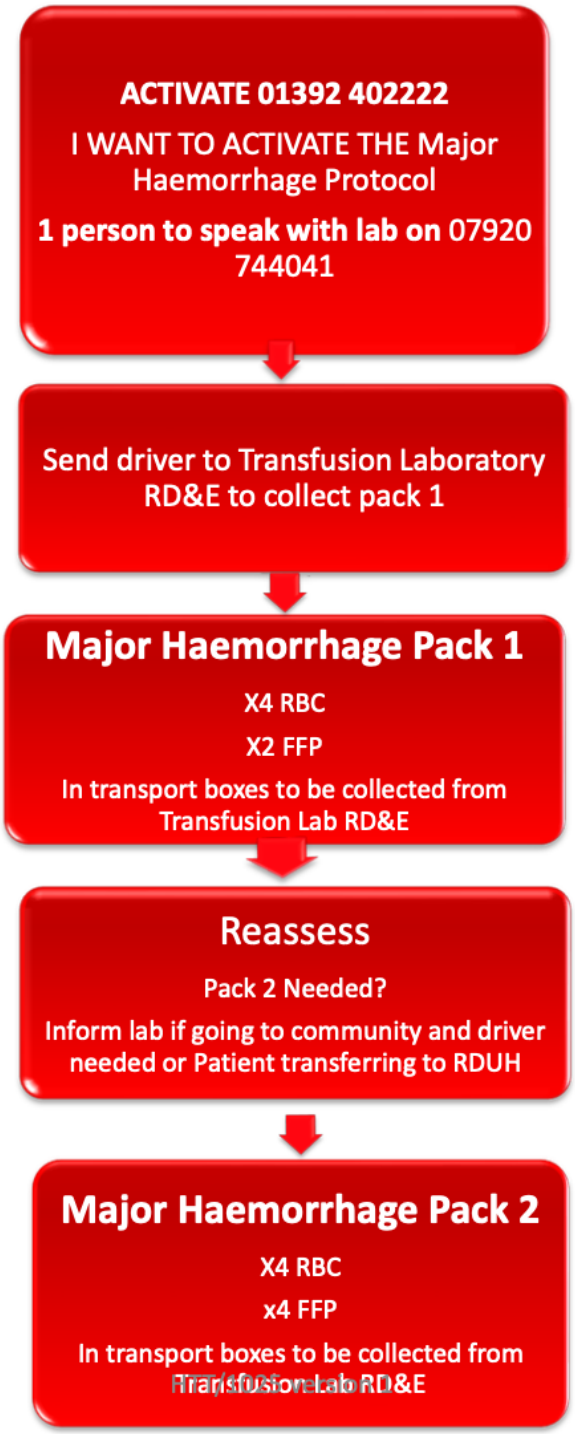
- Ideally ODP bleep holder identifies ODP who can travel with consultant anaesthetist to assist in community hospital (where feasible).

- ODP facilitates gathering equipment (see above) and meets at oncology entrance with equipment.

- Despite this being an emergency the journey must follow Highway Laws

Please note blood components will be sealed in a transport box. If they are unused and remain sealed, they can potentially be reused at the RD&E. If the seal is broken, they must be discarded by the laboratory as they have been outside the fridge too long. The box, and any contents including any unused components, must be returned to the transfusion laboratory as soon as possible. Do not dispose of unused components at the community site.

All blood components must be accounted for, this is a legal requirement. BloodTrack Tx must be used for electronic traceability for components transfused. If BloodTrack Tx cannot be used the patient details, date and time of transfusion must be recorded on the compatibility label, to be returned to the transfusion laboratory, and the component unique number recorded in the EPIC notes.

MAJOR HAEMORRHAGE - COMMUNITY

Blood Transfusion (Eastern) Laboratory Flowchart